



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه بهاء عبدالغفار عبدالهادي

المحافظة : القاهرة الحي / المدينة : بولاق ابو العلا

رقم البطاقة : 27811160103469 رقم التليفون : 01155328437

العمر (تاريخ الميلاد) : 16-11-1978 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر بالصبغه [11127]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمة بهاء عبدالغفار عبدالهادي
National ID: 27811160103469



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest and abdomen

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جولف
روستني
روستني

9-12



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر

09/13/2026

MDT FORM

Name:	لطيفه محمد جلال محمد جلال	MRN:	13059
Age:	48 Y	ID:	27811160103469
Menopausal Sta	PRE	Phone:	01155328437
NRS SCORE:		Marital Status:	N
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

Clinical Data:

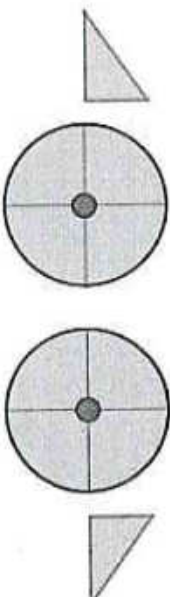
BX:	IDC G3								
ER:	8/8	PR:	0/8	HER2:		SISH NO1		KI67:	

R

L

Sonomammography:

CEM: LEFT BREAST UOQ SPICULATED LESION WITH NON MASS COLLECTIVELY MEASURING 11*5 CM
NORMAL SKIN THICKNESS
NORMAL MAC
NO PATHOLOGICAL AXILARY LNS
BX: IDC G3 ER 8/8 PR 0/8 HER2 +2 SISH: NOT AMPLIFIED
CLINICALLY : LEFT BREAST UOQ LARGE MASS WITH SKIN ATTACHMENT



Mets Workup:
PET CT : NO OTHER FDG ACTIVE LESIONS

T N M

History:

Recommendations:

MDT Outcome & Plan

FOR CT CAP IN GRI
PORT-CTH
ECHO
CLIPPING + BRACKTING
NA CTH FOR DOWNSIZING AC X4 TTX 4 US EVERY 2 CYCES
THEN WLE+LN ACCORDING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
2 محمد كمال	2 محمد عبد الله لوي				

2 محمد كمال

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
بهاء عبدالغفار عبدالهادي
٣٤ شارع العتلات
بولاق - القاهرة

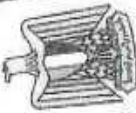
١٩٧٨/١٧٨٦

٢٧٨١١١٦٠١٠٣٤٦٩

IL5802552



٢٠٢٠/٧



٢٧٨١١١٦٠١٠٣٤٦٩
مخاضيه حره

مسلمة
متروجة
مضى الدين شعبان احمد عبدالهادي النجار
البطاقة سارية حتى ٢٠٢٧/٧/٠٧



Patient Name	فاطمة بهاء عبدالغفار	Case No.	357695
Sex/Age	Female 47 Y	Patient No.	25811
Registered Date	12/8/2026 01:18:16 PM	Referred By Dr.	Out Patient
Validation Date	06/9/2026 12:43:08 PM	Branch	Gustave Roussy
Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

Breast Slide Revision

Clinical Data: Left breast 2 O'clock lesion (BIRADs 4c).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (2339126). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2& SISH).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): Moderate.

IPT:

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Positive (8/8).
- PR: Negative (0/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group II.

(Average Her2 copy No.: (3.05), Her2/CEP17 ratio: (2.7)).



شركة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

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Reporting Date	09/9/2026 10:00:08 AM	Pathology No.	SR-938/26

****Scoring system: "ER, PR"**
 ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 10%; 3, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, and 5, between 67%-100% of the cells staining). Total score from 0 to 8.

****Scoring system: "Her2"**
 Reference: ASCO-CAP Her2 Test Guideline Recommendations.IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the

the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells. invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4. (Recounting
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.
- 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



خريطة رقم (4) - الشور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

جمهورية مصر العربية
بطاقة تحفيق الشعب

فاطمة
بهاء عبدالغفار عبدالهادي
٣٤ شارع العائلات
بولاق - القاهرة

١٩٧٨/١ ٢٧٨ ١١ ١٦ . ١ . ٣٤ ٦٩

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٢٠٢٠/٠٧

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مقامية حره



متزوجة
مسلمة
انثى
محي الدين شعبان احمد عبدالهادي النجار
البطاقة سارية حتى ٢٠٢٧/٠٧/٠٧



REPRODUCED BACK

BAGHEYA

Radiology Department

قسم الأشعة التشخيصية

PATIENT NAME: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY

PATIENT ID: 10126282501

GENDER: Female

AGE: 47 y

DATE : 04-Jul-2026 01:25 PM

ORDER: 3939421,3939422

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Heterogeneously dense breasts (ACR c).

Left breast:

- Left UOQ posterior third dense spiculated mass lesion seen measuring 5.2x4.7cm, 10 cm distance from nipple at CC view.
- Complementary US shows 2:00 zone c hypoechoic vertically oriented spiculated mass lesion seen about 2mm deep to overlying skin.
- No pathologically enlarged axillary level I, II or III lymph nodes.
- Clear supraclavicular regions bilaterally.

Right breast:

- No spiculated mass lesions or grouped microcalcifications.
- UOQ two nearby parallel hypoechoic circumscribed lesions at 10:00 measuring 1.6x0.4 and 1x0.6cm.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Left breast UOQ malignant looking lesion (BIRADS 4); for histopathological correlation.
- Right breast UOQ benign looking nodules (BIRADS 3).



Referral to clinic

Dr. Ghada Ali, MSc

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 1, 04-Jul-2026 02:52 PM



1602

MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: External Clinic
D.O.B: 16/11/1978
Sex: Female

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2607060199
Accession No SUR-26-2339
Sample Type Tissue
Collection Date 06/07/2026 01:22 PM
Received Date 06/07/2026 01:22 PM
Doctor Zeinab Amr Mostafa Kamal

Clinical Information

Left breast lesion at 2 O'clock (BIRADS 4c).

Gross Picture

Multiple grayish tissue cores 0.8-1.3 cm long, totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 3. No DCIS component. scattered lymphovascular emboli are detected . Stroma is desmoplastic with mild inflammatory cell reaction.

Frozen Diagnosis

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Left

Specimen type:

U/S guided core biopsy.

Location

2 O'clock

Lesion Size

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

3

M

3

Present

Invasive duct carcinoma, grade 3.

ER, PR & HER-2 are available (upon request).

B5b= Invasive cancer

Dr. Ahmed Ali. MD.

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Marwa Mahmoud Abdelkader Nada

Page Number: 2 of 2

MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: Surgical Oncology

D.O.B: 16/11/1978
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2607130195
Accession No IMM-26-2798
Sample Type Paraffin Block

Collection Date 13/07/2026 12:12 PM
Received Date 13/07/2026 12:12 PM
Doctor Maha Mohamed Wagdy Omar

Immunohistochemical Report

pathology number
2339/26

Clinical Data
Left breast cancer

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong).
Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

ALHADAY.

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
left

Diagnosis :

Left breast cancer:

ER : positive, strong nuclear reaction in 100% of tumor cells (8/8).

PR : Negative (0/8)

Signature

Esraa Esmat, MD

Reported and electronically endorsed by

Esraa esmat mohamed abdelmonem mohamed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Validated on 18/07/2026 07:19 PM

Print Date: 23/07/2026 11:51 AM

Page Number: 2 of 2

MRN: 10126282501
Patient Name: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
IC No.: 27811160103469
Location: Surgical Oncology

D.O.B: 16/11/1978
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2607130194
Accession No IMM-26-2797
Sample Type Paraffin Block

Collection Date 13/07/2026 12:12 PM
Received Date 13/07/2026 12:12 PM
Doctor Maha Mohamed Wagdy Omar

Immunohistochemical Report

pathology number
2339/26

Clinical Data

Left breast cancer

Gross Picture :

Referred block submitted for HER 2 immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:

left

Laterality :

Left

Her2 Result

Equivocal [score 2+]. Recommended For SISH

ALHADAY.

Comment :

STSH is available upon request

Signature

Esraa Esmat, MD

Reported and electronically endorsed by

Esraa esmat mohamed abdelmonem mohamed

**Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP**

Validated on 18/07/2026 07:20 PM

Print Date:

23/07/2026 11:51 AM

Page Number: 2 of 2

PATIENT NAME: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY
GENDER: Female
DATE : 21-Jul-2026 10:14 AM

PATIENT ID: 10126282501
AGE: 47 y
ORDER: 3951711

WHOLE BODY FDG PET-CT EXAMINATION

CLINICAL BACKGROUND:

- 47 year-old female patient
- Presented with left breast cancer
- Weight = 83 Kg
- BL glucose level = 113 mg/dl

REASON OF STUDY:

- Initial staging study

TECHNIQUE:

- Approximately one hour after IV injection of the F-18-FDG 5.8 mCi, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUVmax)~ 3.52

FINDINGS:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- Left breast metabolically active (FDG avid) UOQ irregular soft tissue mass is seen measuring about 4.1 x 3.9 cm, SUVmax~ 17.02. It is seen abutting the skin with minimal overlying skin thickening.
- Few small left axillary metabolically inactive lymph nodes the largest measuring 1.6 x 0.6 cm.
- Right breast no FDG avid focal sizable nodules/masses.

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y

Report version 3, 22-Jul-2026 11:02 AM

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16602

مركز هية النوى
4 شارع علوية اول شارع الهرم جوار دلتا، القاهرة
مستشفى هية النوى
4- ابي الأول - الشيخ زايد رقم 1 كنية الجديدة



- No significant FDG uptake in right axillary lymph nodes.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) cervical lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aerodigestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- Heterogenous thyroid gland showing multiple small bilobar low grade metabolically active hypodense nodules largest at left measuring 1.2 x 0.8 cm, SUVmax~ 3.58.
- Right maxillary and sphenoid sinusitis.

III. Chest:

- No metabolically active (FDG avid) pulmonary nodules.
- No metabolically active mediastinal or hilar lymph nodes.
- No metabolically active pleural effusion or masses.
- Average cardiac size. No pericardial effusion detected.

IV. Abdomen & pelvis:

- The liver is average in size with homogenous CT texture. No definite metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of the pancreas, both kidneys and both suprarenal (on non contrast CT basis) with no FDG avid focal lesions.
- No metabolically active abdominal or pelvic lymphadenopathy.
- Under filling of the urinary bladder.
- Right adnexal low grade metabolically active simple cyst measuring 2.3 x 1.6 cm, SUVmax~ 4.19 likely functional.
- IUCD is seen.
- No ascites.

V. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions.
- Left sacral alium and left femoral head small metabolically inactive sclerotic lesions likely bone islands.
- Spondylodegenerative changes with sclerosis of the VEPS likely

ATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 3, 22-Jul-2026 11:02 AM

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16602

مركز باهية
4 شارع مدينة أول شارع الوحدة بجوار نادي الجزيرة
مستشفى باهية الشيخ زايد
ع. الفاي الأول - الشيخ زايد - ع. الفاي الأولى



degenerative.

OPINION:

FDG-PET/CT study of pathologically proven left breast cancer at initial staging showing:

- Left breast metabolically active neoplastic lesion.
- No other metabolically active (FDG avid) lesions in the rest of the surveyed body.

Reported By,

Dr. Ali Saber, MSc

Dr. Salwa Yahya, MD

Prof. Mai Amr, MD

22-Jul-2026 11:02 AM

PATIENT: FATMA BAHAA ABD EL GHAFAR ABD ALHADAY, 10126282501, Female, 47 y
version 3, 22-Jul-2026 11:02 AM

Medical Department
الطبية
الطبية

Page 3 of 3



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16602

مركز خدمة العملاء
4 شارع طلوع الشمس شارع الفتح جوار نادي البحيرة
مستشفى مدينة الشيخ زايد
ع. قاضي الأول - الشيخ زايد أمام كلية الهندسة



PATIENT NAME: FATMA BAHAA ABD EL GHAEFFAR ABD ALHADAY

GENDER: Female

DATE : 13-Jul-2026 01:47 PM

PATIENT ID: 10126282501

AGE: 47 y

ORDER: 3951690

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

INDICATION:

- Left carcinoma

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV low osmolar non-ionic contrast medium infusion 100 ml

REPORT:

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Left UOQ spiculated mass is seen showing marked heterogeneous mass enhancement measured 5.8x3.5 cm, anterior and inferior enhancing nodules and mass enhancement are seen .The mass & non mass are distributed over an area measuring 11x5 cm ,the process being 4 cm from the nipple
- No right breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

OPINION:

- Left breast UOQ multifocal carcinoma (BIRADS 6).
- No right breast pathological mass or non mass enhancement (BIRADS 1)

Reported By,

Dr. Rasha Wessam, MD

13-Jul-2026 02:33 PM

PATIENT: FATMA BAHAA ABD EL GHAEFFAR ABD ALHADAY, 10126282501, Female, 47 y
Report version 1, 13-Jul-2026 02:33 PM



جستار
روشنی
روشنی



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر

09/13/2026

MDT FORM

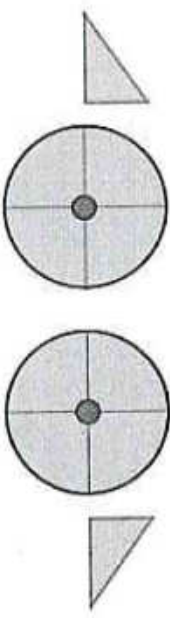
Name:	اسم المريض	MRN:	13059
Age:	48 Y	ID:	27811160103469
Menopausal Sta	PRE	Phone:	01155328437
NRS SCORE:		Marital Status:	N
BMI:		Family History:	
CO Morbidity:	FREE	Reason Of MDT:	NEW CASE

Clinical Data:

BX:	IDC G3	R	L
ER:	8/8	PR:	0/8
		HER2:	
		SISH NO1	
		KI67:	

Sonomammography:

CEM: LEFT BREAST UOQ SPICULATED LESION WITH NON MASS
COLLECTIVELY MEASURING 11*5 CM
NORMAL SKIN THICKNESS
NORMAL MAC
NO PATHOLOGICAL AXILLARY LNS
BX: IDC G3 ER 8/8 PR 0/8 HER2 +2 SISH: NOT AMPLIFIED
CLINICALLY : LEFT BREAST UOQ LARGE MASS WITH SKIN ATTACHMENT



Mets Workup:
PET CT : NO OTHER FDG ACTIVE LESIONS

TNM

History:

Recommendations:

MDT Outcome & Plan

FOR CT CAP IN GRI
PORT-CTH
ECHO
CLIPPING + BRACKTING
NA CTH FOR DOWNSIZING AC X4 TTX 4 US EVERY 2 CYCES
THEN WLE+LN ACCORDING

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
اسم الطبيب	اسم الطبيب				